



# Individual Contribution Statement

*MedGuardian AI — Management of AI Products (MBACC ZG583)*

*2024BC26670 – Sangitha V*

## Team & Roles

*Individual submission.*

| Name       | Role        | Primary sections / artifacts owned                                                                                                                                                                                                                                                            |
|------------|-------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Sangitha V | Sole Author | Full product proposal (13 sections, incl. Specialty Pharma Extension and Regulatory Positioning); MVP 1 codebase, tests, and docs; three presentation decks (prototype review, capabilities/reuse, full overview) plus the MVP 1 spec; src_docs phase documentation and architecture diagram. |

## What I Personally Decided vs. What AI Drafted

### Content I directed but AI drafted

All AI product controls: direction, prompting, and review of the product proposal, MVP 1 codebase, presentation decks, and Phase 2 preview features were done by myself.

### Content I wrote or substantially edited myself

All repository content (code, docs, decks) was AI-drafted under my direction and review; my own contribution took the form of direction, feature and bug diagnosis from actually using the running app (for example, identifying the unreadable FDA interaction text, the missing per-drug check-in, and the missing regimen view), and scoping decisions rather than hand-written prose or code.

## What I'd Do Differently

Before treating this as more than a demo scaffold, I would validate two things with an actual pharmacist: the "regimen role" tags (first-line vs. add-on) and the seven-drug pairwise interaction table. I made up both as placeholders, neither comes from a clinical reference. The pairwise interaction data still isn't a licensed source like DrugBank or FAERS. I added openFDA label-text summarization to work around that gap, but it only fixes readability, not the missing license. The Phase 2 AI layer (summarization and the chatbot) also needs real output review before it reaches an actual patient. Right now, its only safety net is telling the user to ask a pharmacist. That was a reasonable boundary for an MVP preview, but it isn't enough for production. I also want to flag how I made the decision to wall the chatbot off from the Phase 1 core: I made that call reactively, right after I saw the unreadable interaction text myself, not because I planned it from the start. The biggest open risk follows this: MVP 1's safety claims are only as strong as its zero-AI Phase 1 core, and the Phase 2 preview still needs its own separate validation before it earns that same trust.

Drawing on my experience with patient analytics, I believe this application could serve as a bridge for patients to track their medication regimen on a daily basis. If the application succeeds in Phase 1, it could progress to Phase 2 and pursue classification as Software as a Medical Device (SaMD), a regulatory approval process that could take around one to two years.

## Time Investment

| Activity                                   | Approximate time                                                                                                                                                                                                                                                                                                              |
|--------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Strategy &amp; proposal development</b> | About 24 hours, including time spent connecting with the marketing team to brainstorm before reviewing the existing code and product proposal, working through feature ideas like the auto-message/family-branching flow, and figuring out what was realistically doable for Phase 1 before committing to a build.            |
| <b>Prototype development &amp; testing</b> | About 40 hours, where most of the effort went, restructuring MVP 1 after studying a peer example's engineering pattern, pulling in real FDA data, and then iterating the dashboard multiple times based on actually using it (medication remove/reset, per-drug check-ins, the regimen view, and the AI summarization layer). |
| <b>Documentation &amp; presentation</b>    | About 15 hours, putting together several presentation decks, the full src_docs write-up, the architecture diagram, and this statement.                                                                                                                                                                                        |

*Total: approximately 79 hours.*

**Signed: Sangitha V, 16 August 2026**